Population Health Policy: Chronic Disease Screening, Recall and Programme Interventions

National Health Authority (Fictional) · Document NHA-PP-01 · Version 2.1 · Issued June 2026 · Synthetic Demonstration Edition

§1 Purpose

This policy defines the population-health screening, recall, and intervention standards for the national chronic-disease programme, and the approved programme interventions with their expected effects. It is the reference document for population-level planning and policy simulation.

§2 Care gap definitions

- HbA1c test overdue: more than 6 months since the last HbA1c result.
- Retinal exam overdue: more than 12 months since the last retinal examination.
- Foot exam overdue: more than 12 months since the last foot examination.
- Statin not prescribed: High or Very High cardiovascular risk without an active statin prescription.
- BP above target: most recent systolic pressure 140 mmHg or above.
- Anticoagulation review needed: atrial fibrillation without a completed stroke risk assessment.

§3 Recall standards

- Every patient with one or more open care gaps must receive a recall contact within 30 days of the gap opening.
- Recall messages must be available in Arabic, English, Urdu, Hindi, Malayalam, and Tagalog.
- Facilities with more than 25% of their panel carrying two or more open gaps are flagged for a supervised improvement plan.

§4 Equity monitoring

- Control rates, screening coverage, and admission rates must be reported separately for Nationals and Residents, and by emirate, every quarter.
- A difference of more than 5 percentage points in well-controlled rate between population groups triggers an equity review.

§5 Approved programme interventions and expected effects

The following interventions are approved for population-level planning. Expected effects are programme planning assumptions derived from published international programme evaluations, stated per 12 months of operation.

- Intensive case management (top 10% risk): expected mean HbA1c reduction of 0.6 percentage points and 22% fewer admissions among enrolled patients; indicative cost AED 1.9M per 1,000 enrolled.
- GLP-1 access programme (eligible uncontrolled Type 2 with BMI above 30): expected mean HbA1c reduction of 0.9 percentage points and 28% fewer admissions among treated patients; indicative cost

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AED 3.4M per 1,000 treated.

- Statin gap closure programme: expected mean LDL reduction of 1.1 mmol/L and 24% relative reduction in cardiovascular events among treated patients; indicative cost AED 0.9M per 1,000 treated.
- Multilingual recall campaign: expected 40% reduction in overdue screening gaps and 8% fewer admissions across the contacted population; indicative cost AED 0.3M per 1,000 contacted.

Combined interventions must not simply sum expected effects; apply a 25% overlap discount when two or more interventions target the same cohort.

§6 Governance of analytical models and AI agents

- Any predictive model used for patient prioritisation must be registered in the national model inventory with its training data description and last validation date.
- AI agents answering population queries must cite the policy or guideline section supporting any clinical statement and must not expose patient-level records to users without a clinical role.
- Patients with Restricted consent status are excluded from patient-level AI retrieval; they remain in de-identified aggregate statistics.
- All AI-drafted clinical actions require human approval as per NHA-CG-01 §7.

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